

BEAPEN VK GRANULES 125MG/5ML

DESCRIPTION

An almost white powder when reconstituted forms a red fruity flavoured, homogenous suspension. Each 5 ml contains Phenoxymethylpenicillin 125 mg.

INDICATIONS

Phenoxymethylpenicillin is used in the treatment and prophylaxis of infections caused by susceptible organisms, especially streptococci. It is used only for the treatment of mild to moderate infections, and not for chronic, severe, or deep-seated infections. Specific indications for phenoxymethylpenicillin include otitis media, pharyngitis or tonsillitis, primary and secondary prophylaxis of rheumatic fever, skin infections. Other indications include bacterial exacerbations of bronchitis prophylaxis of diphtheria and bacterial endocarditis, erysipelas, gingivitis, scarlet fever and sinusitis.

PHARMACODYNAMICS

Phenoxymethylpenicillin is bactericidal and acts by inhibiting bacterial cell wall synthesis. Bacterial cell walls are held rigid and protected against osmotic rupture by peptidoglycan. Phenoxymethylpenicillin inhibits the final crosslinking stage of peptidoglycan production by binding to and inactivating transpeptidases, penicillin-binding proteins on the inner surface of the bacterial cell membrane. Other mechanisms involved include bacterial lysis by the inactivation of endogenous inhibitors of bacterial autolysins.

PHARMACOKINETICS

Phenoxymethylpenicillin is rapidly, but variably absorbed from the gastrointestinal tract following oral administration. Peak plasma concentrations of 3-5 mcg/ml have been observed 30-60 minutes after a dose of 500 mg. The effect of food on absorption appears to be slight in general. Its half-life is about 30-60 minutes and may be increased to 4 hours in severe renal failure. About 80% is protein bound. Phenoxymethylpenicillin is widely distributed in body tissues and fluids. A high concentration of the drug is achieved in the peritoneal fluid, blister fluid and urine. Distribution into the cerebrospinal fluid (CSF) is low in subjects with noninflamed meninges. It crosses placenta into foetal circulation, and small amounts appear in breast milk. Phenoxymethylpenicillin is metabolised to some extent to penicilloic acid. The unchanged drug and metabolites are excreted rapidly in the urine. Only small concentrations are excreted in the bile.

DOSAGE

ROUTE OF ADMINISTRATION: Oral

Adults

Antibacterial : 250-750 mg every 6 hours.

Erysipelas : 500 mg every 6 hours.

Gingivitis : 500 mg every 6 hours.

Pasteurella infections and rat-bite fever : 500mg every 6 hours for 10-14 days.

Prophylaxis against rheumatic fever : 125 – 250mg twice daily.

Usual adult prescribing limit : up to 7.2 g daily.

Children

Antibacterial : 6 -12 years 250 mg; 1-5 years 125 mg; up to 1 year 62.5 mg. To be administered 6 hourly.

Lyme disease : 5-12.5 mg/kg body weight 4 times daily for 3-4 weeks. Duration of treatment is based on clinical response.

OVERDOSAGE

There is no specific antidote for the treatment of phenoxymethylpenicillin overdose. Treatment should be symptomatic and supportive. Hemodialysis may aid in the removal of penicillins from the blood.

Serious anaphylactoid reactions require immediate emergency treatment with parenteral epinephrine, oxygen, intravenous corticosteroids and airways management (including intubation).

For *Clostridium difficile* colitis, mild cases may respond to discontinuation of the medicine alone. Moderate to severe cases may require fluid, electrolyte and protein replacement. In cases not responding to the above measures, oral doses of vancomycin, metronidazole or cholestyramine may be used.

Vancomycin : 125 mg 6 hourly for 5-10 days.

Metronidazole : 250 – 500 mg 8 hourly.

Cholestyramine : 4 g four times daily.

If diarrhoea occurs, administration of antiperistaltic antidiarrhoeals (opioids, diphenoxylate and atropine combination, loperamide) is not recommended since they may delay the removal of toxins from the colon, thereby prolonging and/or worsening the condition.

Up-to-date information on treatment of overdose can be obtained from The National Poison Centre, University Sains Malaysia.

TOXICOLOGY

Carcinogenicity and Mutagenicity

No documented long term studies in animals that evaluate the carcinogenic and mutagenic potential of phenoxymethylpenicillin were available.

Pregnancy and Reproduction (FDA Pregnancy Category B)

Reproductive studies performed in the mouse, rat and rabbit given phenoxymethylpenicillin have revealed no evidence of impaired fertility. Phenoxymethylpenicillin crossed the placenta. Adequate and well-controlled studies in humans have not been done to determine whether penicillins are teratogenic; however, penicillins are widely used in pregnant women and problems have not been documented.

Breast-feeding

Penicillins are distributed into breast milk. Although significant problems in human have not been documented, the use of penicillins by nursing mothers may lead to sensitisation, diarrhoea, candidiasis and skin rash in the infant.

Pediatrics

Penicillins have been used in pediatric patients and no pediatrics-specific problems have been documented to date. However, the incompletely developed renal function of neonates and young infants may delay the excretion of the drug.

Geriatrics

Penicillins have been used in geriatric patients and no geriatric-specific problems have been documented to date. However, elderly patients may be more likely to have age-related renal function impairment, which may require and adjustment in dosage in patients receiving this drug.

Dental

Prolonged used of phenoxymethylpenicillin may lead to the development of oral candidiasis.

SIDE EFFECTS

Phenoxymethylpenicillin is generally well tolerated. Gastrointestinal effects like diarrhoea and nausea are common following oral administration of the drug. Headache, oral candidiasis (sore mouth and/or tongue, white patches in the mouth and/or on tongue) and vaginal candidiasis (vaginal itching and discharge) have been reported.

Allergic reactions, specifically anaphylaxis (fast and irregular breathing, puffiness and swelling around the face, shortness of breath, sudden, severe decrease in blood pressure), exfoliative dermatitis (red scaly skin), serum sickness-like reactions (skin rash, joint pain, fever), skin rash, hives or itching occasionally occurs.

Rare adverse effects reported include interstitial nephritis (fever, possible decrease urine output, skin rash), leukopenia and neutropenia (sore throat and fever). Pseudomembranous colitis and *Clostridium difficile* colitis (severe abdominal or stomach cramps and pain, abdominal tenderness, watery and severe diarrhoea which may be bloody, fever), which may occur during therapy or up to several weeks after discontinuation of the drug have also been reported. Seizures are more likely to occur in patients receiving high doses of the drug and/or patients with severe renal function impairment.



Black 100%

CONTRAINDICATIONS

Hypersensitivity to penicillins.

DRUG INTERACTIONS

Since bacteriostatic drugs may interfere with the bactericidal effect of phenoxymethylpenicillin in situations where a rapid bactericidal effect is necessary, it is best to avoid concurrent therapy with chloramphenicol, erythromycins, sulphonamides or tetracyclines.

There have been reports of reduced oral contraceptive effectiveness in women taking phenoxymethylpenicillin resulting in unplanned pregnancy. Patients are advised to use an alternative or additional method of contraception. Phenoxymethylpenicillin may decrease clearance of methotrexate resulting in methotrexate toxicity. Probenecid decreases renal tubular secretion of phenoxymethylpenicillin and may increase risk of toxicity.

PRECAUTIONS

Patients allergic to one penicillin may be allergic to other penicillins. Patients allergic to cephalosporins or cephamycins may be allergic to penicillins also. Desensitisation is necessary if treatment with penicillin is essential. Penicillins should be given with caution to patients with a history of allergy, especially to drugs or history of sensitivity to multiple allergens and patients with impaired renal function.

Renal and hematological status should be monitored during prolonged and high-dose therapy.

Care is also necessary when treating patients with syphilis due to risk of Jarisch-Herxheimer reactions. Contact with skin should be avoided since skin sensitisation may occur. Supra-infection with penicillin-resistant organisms including *Pseudomonas* or *Candida* may occur with prolonged use. Penicillins may cause pseudomembranous colitis in patients with history of gastrointestinal disease, especially antibiotic-associated colitis. Leukopenia or neutropenia is more likely to occur with prolonged therapy and severe hepatic function impairment. High urinary concentrations of penicillin may produce false-positive or falsely elevated test results with copper sulphate tests (Benedict's, Clinitest or Fehling's). Glucose enzymatic tests (Clinistix or Testape) are not affected. False-positive result may occur during therapy with any penicillin in the direct antiglobulin (Coombs') tests. Physiology/laboratory test values of alanine aminotransferase (ALT[SGPT]), alkaline phosphatase (ALP), aspartate aminotransferase (AST[SGOT]) and lactate dehydrogenase (LDH) may be increased.

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Beapen VK Granules, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Beapen VK Granules must be discontinued immediately and appropriate alternative therapy instituted.

PRESENTATION

60ml, 60ml x 5 bottles. (not all pack sizes may be available in the market)

STORAGE CONDITIONS AND USER INSTRUCTIONS

Keep container tightly closed (loose pack and granules only).
Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.
Jauhkan daripada kanak-kanak.
Shelf life: Please refer to outer package.
For granules only : Mix with water before taking. Shake well before each dose. Store tightly closed in refrigerator (2-8°C) after reconstitution. Discard 7 days after reconstitution.
Take at regular intervals. Complete the prescribed course unless otherwise directed.
Take on empty stomach.
CONSUME WITHIN ONE MONTH AFTER OPENING THE POUCH

Product Registration Holder & Manufacturer:

Duopharma (M) Sdn. Bhd.

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